

Schizophrenia - Self-Care

CCG: C-0060 (CCG)

[Link to Codes](#)**MCG Health**

Chronic Care

28th Edition

- Assessment
 - Initial Consideration
 - Condition Knowledge
 - Provider Information
 - Medication Information
 - Complications
 - Monitoring and Management
 - Other Health Considerations
 - Patient Understanding Evaluation
- Quality Measures
- Plan of Care
- Evidence Summary
- References
- Footnotes
- Codes

Assessment

Initial Consideration

Q1. Case Manager: Are there any barriers preventing patient participation in the program? (Check all that apply.) **QM**

- Behavioral or cognitive problems (eg, inappropriate behaviors or very poor memory) Behavioral Health and Cognition Evaluation with Caregiver [CCG](#)
- Hearing problems Hearing [CCG](#)
- Language barrier Language [CCG](#)
- Vision problems Vision [CCG](#)
- No
- Unknown [P1]

Q2. Patient: What would you like to talk about today? (Check all that apply.) **QM**

- Emergency or urgent problems (eg, social determinants of health, intimate partner violence, suicidal ideation)[A] Social Determinants of Health Screening Assessment [CCG](#) Intimate Partner Violence [CCG](#) Suicide or Personal Harm Risk [CCG](#)
- Condition information
- Provider coordination (eg, next appointment, treatment clarification) Appointment Scheduling [CCG](#)
- Medications
- Complications
- Monitoring and management
- Other health problems (eg, comorbid conditions, new symptoms)
- Other, specify: _____
- Nothing
- Unknown

Q3. Patient: Do you have any changes since the last time we talked? (Check all that apply.) **QM**

- Adhering to treatment or medication problems Readiness to Change [CCG](#) Medication Adherence [CCG](#)
- Administering medication Home Infusion Management [CCG](#) Injectable Medications [CCG](#)
- Affording medicine or supplies Financial Status and Benefits [CCG](#)
- Aspiration Aspiration Risk Management [CCG](#)
- Behavioral health provider appointment, specify: _____ Follow-Up - Provider Appointment [CCG](#)
- Blood pressure care Blood Pressure Monitoring [CCG](#)

- Blood sugar care Blood Glucose Monitoring [CCG](#) HbA1c Monitoring [CCG](#)
- Blood thinner care Anticoagulation Management [CCG](#)
- Bowel care Bowel Management [CCG](#)
- Bowel changes Bowel Symptoms [CCG](#)
- Caregiver Caregiver Resources Evaluation [CCG](#)
- Chemotherapy Chemotherapy [CCG](#)
- Cognitive impairment Cognitive Impairment [CCG](#) Behavioral Health and Cognition Evaluation with Caregiver [CCG](#)
- Communication or speech problems Speech and Communication Problems [CCG](#)
- Community resource Community Resources [CCG](#)
- Dehydration care Dehydration Monitoring and Management [CCG](#)
- Depression symptoms Depression Screening, Adult [CCG](#)
- Dialysis care Dialysis Management [CCG](#)
- Doctor or specialist outpatient appointment, specify: _____ Follow-Up - Provider Appointment [CCG](#)
- Energy level changes Energy Level Changes [CCG](#)
- Enteral nutrition care Enteral Nutrition Management [CCG](#)
- Exercising and staying active Exercise [CCG](#)
- Fatigue or weakness Fatigue or Weakness [CCG](#)
- Fluid restriction Fluid Restriction [CCG](#)
- Functional status changes Activities of Daily Living [CCG](#) Instrumental Activities of Daily Living [CCG](#)
- Home infusion Home Infusion Management [CCG](#)
- Hospital or emergency room visit, specify: _____ Hospitalization Risk Assessment [CCG](#) Post-Hospitalization Follow-Up [CCG](#)
- Medication changes Medication Changes [CCG](#)
- Nebulizer care Nebulizer Management [CCG](#)
- Neurologic function changes Neurologic Function Changes [CCG](#)
- Ostomy care Ostomy Management [CCG](#)
- Oxygen care Oxygen Management [CCG](#)
- Pain, chronic Chronic Pain [CCG](#)
- Pain, new or general Pain Assessment [CCG](#)
- Parenteral nutrition care Parenteral Nutrition Management [CCG](#)
- Peak flow meter care Peak Flow Meter Use [CCG](#)
- Peripheral vascular (eg, lower leg redness or swelling) Peripheral Vascular Problems [CCG](#)
- Physical impairments Physical Impairments [CCG](#)
- Planning for long-term care needs Advance Care Planning [CCG](#)
- Procedure, specify: _____ Follow-Up - Provider Appointment [CCG](#)
- Pulmonary treatments Pulmonary Treatments [CCG](#)
- Rehabilitation therapy, specify: _____ Follow-Up - Provider Appointment [CCG](#)
- Respiratory changes Respiratory Symptoms [CCG](#)
- Safety Safety [CCG](#)
- Scheduling care Appointment Scheduling [CCG](#)
- Sleeping problems Sleep [CCG](#)
- Social support Social Support [CCG](#)
- Stasis ulcer care Stasis Ulcer Management [CCG](#)
- Testing (eg, laboratory, imaging), specify: _____ Follow-Up - Provider Appointment [CCG](#)
- Tracheostomy care Tracheostomy Management [CCG](#)
- Transportation Transportation [CCG](#)

- Understanding healthcare needs Health Literacy [CCG](#)
- Urinary catheter care Urinary Catheter Management [CCG](#)
- Urinary changes Urinary Symptoms [CCG](#)
- Venous thromboembolism symptoms Venous Thromboembolism [CCG](#)
- Ventilator care Ventilator Management [CCG](#)
- Weight gain Nutritional Status, Adult [CCG](#) Nutritional Status, Pediatric [CCG](#) Nutritional Status, Infant [CCG](#)
- Weight loss Nutritional Status, Adult [CCG](#) Nutritional Status, Pediatric [CCG](#) Nutritional Status, Infant [CCG](#)
- Wound care Wound Management [CCG](#)
- Other, specify: _____
- No
- Unknown Hospitalization Risk Assessment [CCG](#)

Condition Knowledge

Q1. Patient: What causes schizophrenia? (Check items understood.)

- Actual cause unknown
- Brain chemical imbalance
- Family history (genetics)
- Other, specify: _____
- Unknown [P1]
- Nothing [P1]
- Not all appropriate information known [P1]

Q2. Case Manager: Rate the patient's understanding of the causes of schizophrenia. **QM**

- 90% or more
- 75% to 89% [P1]
- 50% to 74% [P1]
- Less than 50% [P1]
- Unknown [P1]

Q3. Patient: What are the symptoms of schizophrenia? (Check items understood.)

- Confused thinking
- Delusions, like believing that others are controlling your mind or that someone is out to hurt you
- Difficulty having fun
- Difficulty showing emotion
- Difficulty socializing with others
- Feeling suspicious about others
- Feeling very irritable
- Hallucinations, like seeing things that are not there or hearing voices
- Talking that does not make sense
- Trouble completing activities
- Trouble concentrating or paying attention
- Trouble sleeping
- Other, specify: _____
- Unknown [P1]
- Nothing [P1]
- Not all appropriate information known [P1]

Q4. Case Manager: Rate the patient's understanding of the symptoms of schizophrenia. **QM**

- 90% or more
- 75% to 89% [P1]
- 50% to 74% [P1]
- Less than 50% [P1]
- Unknown [P1]

Q5. Patient: How can you care for schizophrenia to keep it from getting worse and causing problems? (Check items understood.) **QM**

- Adhere to self-care plan
- Ask for help when feelings are overwhelming
- Exercise
- Follow a daily care plan

- Follow an eating (diet) plan
- Get all follow-up laboratory tests when due
- Keep a diary about feelings, activities, eating times, and medicines taken
- Keep all doctor appointments
- Keep a vaccination schedule
- Know when to contact the doctor
- Manage situation or event triggers
- Manage stress
- Seek emergency care for signs of crisis plan not working
- Set realistic expectations for feeling better, managing symptoms, and recovery
- Stop using alcohol, tobacco, or drugs
- Take medicines as ordered
- Other, specify: _____
- Unknown [P1]
- Nothing [P1]
- Not all appropriate information known [P1]

Q6. Case Manager: Rate the patient's understanding of how to care for schizophrenia. **QM**

- 90% or more
- 75% to 89% [P1] [P2]
- 50% to 74% [P1] [P2]
- Less than 50% [P1] [P2]
- Unknown [P1] [P2]

Q7. Patient: What activities would you be able to do if you didn't have problems with your schizophrenia? (Check all that apply.) **QM**

- Activities of daily living (ADL) without assistance [P2]
- Drive [P2]
- Go out with friends [P2]
- Go to events [P2]
- Have a relationship [P2]
- Live independently [P2]
- Make and keep friends [P2]
- Play with children or grandchildren [P2]
- Repair broken relationships [P2]
- Shop [P2]
- Play sports [P2]
- Travel [P2]
- Work [P2]
- Other, specify: _____ [P2]
- Unknown [P1]

Provider Information

Q1. Patient: What doctor manages your schizophrenia? **QM**

- Psychiatrist or psychologist, name and phone number: _____ [Q2]
- Behavioral health therapist, name and phone number: _____ [Q3]
- Primary doctor, name and phone number: _____
- Other specialist, name and phone number: _____ [Q5] [P1]
- Don't have doctor [P1]
- Unknown [P1]

Q2. Patient: When was your last visit with the psychiatrist or psychologist?

- Less than 1 month ago
- 1 to 3 months ago
- 4 to 6 months ago [P2]
- More than 6 months ago [P2]
- Unknown [P2]
- Never [P2]

Q3. Patient: When was your last visit with a behavioral health therapist?

- Less than 1 month ago
- 1 to 3 months ago [P2]

- 4 to 6 months ago [P2]
- More than 6 months ago [P2]
- Unknown [P2]
- Never [P2]
- Not applicable

Q4. Patient: When was your last visit with your primary doctor? **QM**

- Within the past 3 months
- 4 to 6 months ago
- More than 6 months ago [P2]
- Unknown [P2]
- Never [P2]

Q5. Patient: When was your last visit to the specialist doctor?

- Within the past 3 months
- 4 to 6 months ago [P2]
- More than 6 months ago [P2]
- Unknown [P2]
- Never [P2]

Q6. Patient: What other specialists have you seen within the last 12 months? (Check all that apply.) **QM**

- Behavioral health provider
- Cardiologist
- Dentist
- Dietitian
- Endocrinologist
- Gastroenterologist
- General surgeon
- Genetic counselor
- Geriatrician
- Gynecologist
- Hematologist
- Hepatologist
- HIV/AIDS
- Infectious disease
- Nephrologist
- Neurologist
- Neurosurgeon
- Obstetrician
- Oncologist
- Ophthalmologist
- Orthopedic surgeon
- Orthopedist
- Otolaryngologist
- Pain management
- Psychiatrist
- Plastic surgeon
- Podiatrist
- Pulmonologist
- Rheumatologist
- Transplant specialist
- Urologist
- Other, specify: _____
- None [P1]
- Unknown [P1]

Q7. Patient: How many schizophrenia-related emergency department visits or hospital admissions have you had in the past 12 months? **QM**

- 2 or more [P2] Hospitalization Risk Assessment [CCG](#)
- 1 [P2]
- None
- Other, specify: _____

- Unknown [P2]

Q8. Patient: Do you have a written self-care plan from your doctor on how to care for schizophrenia?[B] **QM**

- Yes [Q9]
- No [P1] [P2]
- Unknown [P1] [P2]

Q9. Patient: What are the components of your schizophrenia self-care plan? (Check all that apply.) **QM**

- Action plan for when symptoms start
- Attending regular therapy or counseling appointments
- Crisis management plan
- Exercise plan
- Knowing what medicines you take and why and how to take them
- Managing side effects of medicines
- Medicine use
- Nutrition plan
- Recognizing the symptoms of gradual worsening or problems
- Regular follow-up appointments for wellness checks
- When to call the doctor and when to get emergency help
- Other, specify: _____ [P1]
- Unknown [P2]
- Not applicable
- Not all appropriate information known [P2]

Q10. Case Manager: Is the patient able to understand and adhere to the self-care plan? **QM**

- Yes, independently
- Yes, with occasional reminders or caregiver support
- Yes, with frequent reminders or caregiver support
- No [P3]
- Unknown [P3]

Medication Information

Q1. Patient: Has any medicine for schizophrenia been prescribed by your doctor? If so, is it taken as ordered? **QM**

- Yes, and taken as ordered [Q2]
- Yes, but not taken as ordered [Q2] [P4] Medication Adherence  CCG
- No medicine prescribed [P4]
- Unknown [P4]

Q2. Patient: What medicines do you take for schizophrenia? (Check all that apply.) **QM**

- Atypical antipsychotics (eg, aripiprazole, clozapine, risperidone, ziprasidone) [Q3] [P5] [P6]
- Injectable antipsychotics (eg, fluphenazine decanoate, haloperidol decanoate, risperidone) [Q3] [P5] [P7]
- Typical antipsychotics (eg, chlorpromazine, haloperidol, thioridazine) [Q3] [P5] [P7]
- Other, specify: _____ [Q3] [P4] [P5]
- Not applicable [P4]
- Unknown [P5]

Q3. Patient: Are you having any side effects from the schizophrenia medicine? (Check all that apply.)

- Blurred vision [P5]
- Constipation [P5]
- Dizziness [P5]
- Dry mouth [P5]
- Fast heart rate [P5]
- Inability to urinate [P5]
- Involuntary, repetitive movements (eg, grimacing, tongue movements, lip smacking, pursed lips, or excessive eye blinking) [P5]
- Light-headed when getting up from bed or chair [P5]
- Skin rash [P5]
- Tiredness [P5]
- Tremors [P5]
- Upset stomach [P5]
- Weight gain [P5]
- Other, specify: _____ [P5]

- None
- Unknown [P5]
- Not applicable

Q4. Case Manager: Rate the patient's understanding of the purpose and side effects of the medications for schizophrenia. **QM**

- 90% or more
- 75% to 89% [P1] [P5]
- 50% to 74% [P1] [P5]
- Less than 50% [P1] [P5]
- Unknown [P1] [P5]

Q5. Patient: Do you take any medicine that may make your schizophrenia worse? (Check all that apply.)

- Anticholinergics: benztropine, biperiden, or trihexyphenidyl [P3] [P4]
- Bupropion [P3] [P4]
- Carbamazepine (anticonvulsant) [P3] [P4]
- Dopamine [P3] [P4]
- Epinephrine [P3] [P4]
- Oral blood thinners [P3] [P4]
- Paroxetine (antidepressant) [P3] [P4]
- Varenicline [P3] [P4]
- Other, specify: _____ [P3] [P4]
- None
- Unknown [P3] [P4]

Complications

Q1. Patient: What problems with schizophrenia have you had in the past month? (Check all that apply.)

- Believing that others are controlling your mind or that someone is out to hurt you [P2] [P3]
- Feeling confused or not thinking clearly [P2] [P3]
- Feeling that something is crawling on or under your skin [P2] [P3]
- Feeling very irritable [P2] [P3]
- Having trouble concentrating or remembering things [P2] [P3]
- Having trouble walking or moving your arms and legs at the same time [P2] [P3]
- Not able to show emotions [P2] [P3]
- Receiving personal messages from the TV, radio, magazines, posters, newspapers, or other public communications [P2] [P3]
- Seeing things that others cannot see or hearing voices that no one else hears [P2] [P3]
- Other, specify: _____ [P2] [P3]
- None
- Unknown [P2]

Q2. Patient: Has your mood changed over the past 2 weeks?(1)(2)  PHQ-2 Calculator **QM**

- Yes [Q3]
- No
- Unknown

Q3. Patient: What changes in your mood have you had? (Check all that apply.) **QM**

- Change in usual activity (eg, working excessively, no longer talking to friends, or going on shopping sprees) Depression Screening, Adult  CCG
- Down, hopeless, or depressed Depression Screening, Adult  CCG
- Forgetting a lot or getting angry or frustrated Depression Screening, Adult  CCG
- Little interest or pleasure in doing things Depression Screening, Adult  CCG
- Quick mood changes Depression Screening, Adult  CCG
- Other, specify: _____ Depression Screening, Adult  CCG
- Not applicable

Q4. Patient: Are there any current sleeping problems? (Check all that apply.) **QM**

- Falling asleep [P8]
- Staying asleep [P8]
- Waking up too early [P8]
- Not feeling rested after sleeping [P8]

- Other, specify: _____ [P8]
- None
- Unknown [P8]

Q5. Patient: What do you do to help sleep? (Check all that apply.)

- Decrease caffeine
- Don't eat big meals before bedtime
- Get daily exercise
- Go to bed and get up at routine times of the day
- Keep journal of thoughts
- Sleep in bed, not on chair or sofa
- Sleep in dark, quiet room
- Sleep with head elevated, not lying flat
- Stopped drinking
- Stopped smoking
- Take a warm bath or shower before bedtime
- Take medicines, specify: _____ [Q6]
- Took distractions out of bedroom (eg, TV)
- Use equipment (eg, oxygen or CPAP)
- Other, specify: _____
- Nothing
- Unknown [P8]
- Not all appropriate sleep promoting activities being used [P8]

Q6. Patient: Do the medicines work to help sleep?

- Yes
- No [P8]
- Unknown
- Not applicable

Q7. Patient: Are there any safety risks that may lead to problems being safe at home? (Check all that apply.)

- Accessibility (eg, outside or inside stairs) [Q8] [P9]
- Dizziness [Q8] [P9]
- Falls [Q8] [P9]
- Inside home safety hazards (eg, halls and rooms are cluttered, inadequate lighting, bathroom unsafe, stairs without rails, pets) [Q8] [P9]
- Memory or cognition [Q8] [P9]
- Outside home safety hazards (eg, doors that do not lock, stairs without rails) [Q8] [P9]
- Polypharmacy (4 or more medicines) [Q8] [P9]
- Problems with mobility or transfers [Q8] [P9] [Activities of Daily Living](#)  CCG
- Substances used that increase risk (eg, benzodiazepines, sedative-hypnotics, antidepressants, antipsychotics, alcohol, opiates)
- Vision problems [Q8] [P9]
- Other safety concerns: _____ [Q8] [P9]
- None
- Unknown [Q8]

Q8. Patient: Is there a plan in place to manage home safety risks? If yes, what is the plan?

- Yes, specify: _____ [P9]
- No [P10]
- Unknown [P9]
- Not applicable

Q9. Patient: Are you concerned about your safety?

- Yes, specify: _____ [P9] [P10] Intimate Partner Violence  CCG
- No
- Unknown [P9]

Q10. Patient: Has a nurse or physical therapist ever come to your home to do a home safety assessment?

- Yes, specify date: _____
- No [P9]
- Unknown [P9]

Q11. Case Manager: Rate the patient's understanding of the potential complications of schizophrenia. **QM**

- 90% or more
- 75% to 89% [P1] [P2]
- 50% to 74% [P1] [P2]
- Less than 50% [P1] [P2]
- Unknown [P1] [P2]

Monitoring and Management

Q1. Patient: How much do you weigh and how tall are you? **QM**

- Weight: _____
- Height: _____
- Unknown [P11]

Q2. Case Manager: What is the patient's body mass index (BMI)? BMI Calculator

- Less than 18.5 [P11] [P12] Nutritional Management to Gain Weight  CCG Body Weight Planner  CCG
- 18.5 to 24.9
- 25 to 29.9 [P11] [P12] Nutritional Management to Lose Weight  CCG Body Weight Planner  CCG
- 30 or greater [P11] [P12] Nutritional Management to Lose Weight  CCG Body Weight Planner  CCG
- Unknown [P11]

Q3. Patient: Do you know what your body weight should be?

- Yes, it should be: _____
- No [P11]
- Unknown [P11]

Q4. Patient: What do you eat and drink in a typical day? (Check all reported.)

- Alcohol, specify types and number of daily servings: _____ [Q5] [Q6] [Q7] [Q8] [Q9]
- Dairy foods, specify types and number of daily servings: _____ [Q5] [Q6] [Q7] [Q8] [Q9]
- Fats, specify types and number of daily servings: _____ [Q5] [Q6] [Q7] [Q8] [Q9]
- Fruits, specify types and number of daily servings: _____ [Q5] [Q6] [Q7] [Q8] [Q9]
- Fruit juices, specify types and number of daily servings: _____ [Q5] [Q6] [Q7] [Q8] [Q9]
- Grains, specify types and number of daily servings: _____ [Q5] [Q6] [Q7] [Q8] [Q9]
- Meats (beef, chicken, fish), specify types and number of daily servings: _____ [Q5] [Q6] [Q7] [Q8] [Q9]
- Milk, specify types and number of daily servings: _____ [Q5] [Q6] [Q7] [Q8] [Q9]
- Sodas, specify types and number of daily servings: _____ [Q5] [Q6] [Q7] [Q8] [Q9]
- Supplements, specify types and number of daily servings: _____ [Q5] [Q6] [Q7] [Q8] [Q9]
- Sweets, specify types and number of daily servings: _____ [Q5] [Q6] [Q7] [Q8] [Q9]
- Vegetables, specify types and number of daily servings: _____ [Q5] [Q6] [Q7] [Q8] [Q9]
- Water, specify number of daily servings: _____ [Q5] [Q6] [Q7] [Q8] [Q9]
- Other, specify: _____ [Q5] [Q6] [Q7] [Q8] [Q9] [P11]
- No food by mouth [Q9] [P11] [P12]
- Unknown [Q5] [Q6] [Q7] [Q8] [Q9] [P11] [P12]

Q5. Patient: How often do you eat each day? (Check all reported.) **QM**

- 3 meals or more [P11] [P12]
- 2 meals
- 1 meal [P11] [P12]
- 0 meals [P11] [P12]
- 2 snacks or more [P11] [P12]
- 1 snack
- 0 snacks
- Other, specify: _____ [P11] [P12]
- Not applicable
- Unknown [P11] [P12]

Q6. Patient: How much fluid (such as water, juice, coffee, tea, milk) do you have per day? **QM**

- Less than 3 cups (720 mL) [P11] [P12]
- 3 to 5 cups (720 mL to 1.2 L) [P11] [P12]
- More than 5 cups (1.2 L)

- Unknown [P11] [P12]

Q7. Patient: Are you trying to gain or lose weight?

- Trying to gain weight [P11] Nutritional Management to Gain Weight [CCG](#) Body Weight Planner [CCG](#)
- Trying to lose weight [P11] Nutritional Management to Lose Weight [CCG](#) Body Weight Planner [CCG](#)
- Neither
- Unknown [P11]

Q8. Patient: Does your doctor want you to follow a special diet program? (Check all that apply.) **QM**

- Yes, DASH (low sodium) [P11] Diet Knowledge Insufficient - DASH Diet [CCG](#)
- Yes, heart healthy [P11] Diet Knowledge Insufficient - Heart Healthy [CCG](#)
- Yes, kidney diet [P11] Diet Knowledge Insufficient - Renal Dysfunction [CCG](#)
- Yes, low carb or low glycemic index [P11] Diet Knowledge Insufficient - Low Glycemic Index [CCG](#)
- Yes, pregnancy diet [P11] Diet Knowledge Insufficient - Pregnancy [CCG](#)
- Yes, other: _____ [P11]
- No [P11]
- Unknown [P11]

Q9. Patient: Do you get nutritional treatments or have an altered diet? If yes, why? (Check all that apply.) **QM**

- Behavioral health provider seen, specify: _____ [P12]
- Dental soft diet, specify: _____ [P12]
- Diet supplements, specify: _____ [P12]
- Dietitian seen or following, specify: _____ [P12]
- Enteral nutrition (tube feeding), specify: _____ [P12] Enteral Nutrition Management [CCG](#)
- IV fluids (hydration), specify: _____ [P12]
- Liquid diet, specify: _____ [P12]
- Mechanical soft diet, specify: _____ [P12]
- Parenteral nutrition, specify: _____ [P12] Parenteral Nutrition Management [CCG](#)
- Procedure(s) done by a doctor in office, specify: _____ [P12]
- Pureed diet, specify: _____ [P12]
- Speech-language pathology (SLP), specify: _____ [P12]
- Supervised during nutritional intake, specify: _____ [P12]
- Surgery, specify: _____ [P12]
- Thickened liquid diet, specify: _____ [P12]
- Other, specify: _____ [P12]
- No
- Unknown [P11]

Q10. Patient: Do you know how to read food labels?

- Yes
- No [P13]
- Unknown [P13]

Q11. Patient: Do you get regular exercise? **QM**

- Yes
- No
- Unknown

Q12. Patient: Have additional treatments, procedures, or therapy been considered? (Check all that apply.) **QM**

- Electroconvulsive therapy (ECT), date scheduled: _____ [P2]
- Other, specify: _____ [P2]
- None
- Unknown [P2]

Q13. Patient: Do you participate in a program for schizophrenia? (Check all that apply.)

- Inpatient mental health care
- Behavioral health counseling
- Community education program

- Support group
- Exercise program
- Not now, but in the past [Q14] [P2] [P3]
- Other, specify: _____
- No [P2] [P3]
- Unknown [P2] [P3]

Q14. Patient: When was the last time you participated in a therapy program?

- 3 months ago or less
- 4 to 11 months ago
- 12 months ago or more [P2]
- Other, specify: _____ [P2]
- Unknown [P2]
- Not applicable

Q15. Patient: Are you having any problems with your schizophrenia self-care needs? (Check all that apply.) **QM**

- Affording medicine or supplies [P2] [P3] Financial Status and Benefits [CCG](#)
- Behavior indicating cognitive impairment [P2] [P3] Cognitive Impairment [CCG](#) Behavioral Health and Cognition Evaluation with Caregiver [CCG](#)
- Blood pressure care [P2] [P3] Blood Pressure Monitoring [CCG](#)
- Blood sugar care [P2] [P3] Blood Glucose Monitoring [CCG](#)
- Bowel program [P2] [P3] Bowel Management [CCG](#)
- Caregiver help needed [P2] [P3] Caregiver Resources Evaluation [CCG](#)
- Community resource information needed [P2] [P3] Community Resources [CCG](#)
- Daily care of child [P2] [P3] Caregiver Resources Evaluation [CCG](#)
- Exercising and staying active [P2] [P3] Exercise [CCG](#)
- Safety in the home [P2] [P3] Safety [CCG](#)
- Social support [P2] [P3] Social Support [CCG](#)
- Taking medicine as prescribed [P2] [P3] Medication Adherence [CCG](#)
- Transportation for care [P2] [P3] Transportation [CCG](#)
- Trouble gaining weight [P2] [P3] Nutritional Management to Gain Weight [CCG](#)
- Trouble losing weight [P2] [P3] Nutritional Management to Lose Weight [CCG](#)
- Trouble making lifestyle changes [P2] [P3] Readiness to Change [CCG](#)
- Trouble understanding how to do self-care [P2] [P3] Health Literacy [CCG](#)
- Other, specify: _____ [P2] [P3]
- No
- Unknown [P2] [P3]

Q16. Case Manager: Rate the patient's understanding of the monitoring and management of schizophrenia. **QM**

- 90% or more
- 75% to 89% [P1] [P2]
- 50% to 74% [P1] [P2]
- Less than 50% [P1] [P2]
- Unknown [P1] [P2]

Other Health Considerations

Q1. Patient: Do you use tobacco, alcohol, or drugs? (Check all that apply.) **QM**

- Yes, tobacco use [P2] Tobacco Use [CCG](#)
- Yes, alcohol use [P2] [P3] Alcohol Use [CCG](#)
- Yes, drug use [P2] [P3] Substance Use [CCG](#)
- No
- Unknown [P2] Tobacco Use [CCG](#) Alcohol Use [CCG](#) Substance Use [CCG](#)

Q2. Patient: What vaccinations have you had? (Check all that apply.) **QM**

- Coronavirus disease 2019 (COVID-19) [Q3]

- *Haemophilus influenzae* type b (Hib) [Q3] [Q4]
- Hepatitis A [Q3]
- Hepatitis B [Q3]
- Human papillomavirus (HPV) [Q3]
- Influenza [Q3] [Q4]
- Measles, mumps, rubella (MMR) [Q3]
- Meningococcal 4-valent conjugate (MenACWY) or polysaccharide (MPSV4) [Q3]
- Meningococcal B (MenB) [Q3]
- Pneumococcal 13-valent conjugate (PCV13) [Q3] [Q5]
- Pneumococcal 15-valent conjugate (PCV15) [Q3] [Q5]
- Pneumococcal 20-valent conjugate (PCV20) [Q3] [Q5]
- Pneumococcal 23-valent polysaccharide (PPSV23) [Q3] [Q5]
- Polio (inactivated) [Q3]
- Respiratory syncytial virus (RSV) [Q3]
- Rotavirus [Q3]
- Tetanus booster [Q3]
- Tetanus, diphtheria, pertussis (Td/Tdap) [Q3]
- Varicella [Q3]
- Zoster [Q3]
- Other, specify: _____ [Q3]
- None, because: _____ [P14]
- Unknown [P14]

Q3. Patient: Are you up to date with the vaccinations you need (for age, booster status, or medical condition)? **QM**

- Yes
- No, because: _____ [Q6] [P14]
- Unknown [Q6] [P14]

Q4. Patient: When was your last flu shot? **QM**

- In the past year
- More than a year ago [Q6] [P14]
- Never [Q6] [P14]
- Unknown [Q6] [P14]

Q5. Patient: When was your last pneumonia shot?

- In the last year
- 2 to 4 years ago
- 4 to 5 years ago [Q6] [P14]
- More than 5 years ago [Q6] [P14]
- Never [Q6] [P14]
- Unknown [Q6]

Q6. Patient: Do you have an appointment scheduled to get caught up on your vaccinations?

- Yes
- No [P14]
- Unknown [P14]

Q7. Patient: Do you have other conditions that might make schizophrenia worse? (Check all that apply.) **QM**

- Alcohol or substance abuse [P2] [P3] Substance-Related Disorders - Self-Care [CCG](#)
- Anxiety [P2] [P3] Anxiety - Self-Care [CCG](#)
- Bipolar disorder [P2] [P3] Bipolar Disorder - Self-Care [CCG](#)
- Cancer [P2] [P3] Cancer - Self-Care [CCG](#)
- Coronary artery disease [P2] [P3] Coronary Artery Disease - Self-Care [CCG](#) Hypertension - Self-Care [CCG](#)
- Dementia [P2] [P3] Dementia - Self-Care [CCG](#)
- Depression [P2] [P3] Depression - Self-Care [CCG](#)
- Diabetes [P2] [P3] Diabetes, Adult - Self-Care [CCG](#)
- High blood pressure [P2] [P3] Hypertension - Self-Care [CCG](#)
- High cholesterol [P2] [P3] High Cholesterol - Self-Care [CCG](#)

- Kidney disease [P2] [P3] Kidney Disease - Self-Care  CCG
- Liver disease [P2] [P3] Liver Disease - Self-Care  CCG
- Metabolic syndrome [P2] [P3]
- Obesity or overweight [P2] [P3] Obesity, Adult - Self-Care  CCG
- Physical inactivity [P2] [P3] Exercise  CCG
- Posttraumatic stress disorder (PTSD) [P2] [P3] Anxiety - Self-Care  CCG
- Pregnancy [P2] [P3] High-Risk Pregnancy - Self-Care  CCG
- Seizures [P2] [P3] Seizure Disorders - Self-Care  CCG
- Systemic lupus erythematosus [P2] [P3]
- Thyroid disease [P2] [P3]
- Other, specify: _____ [P2] [P3]
- No
- Unknown [P1]

Patient Understanding Evaluation

Q1. Case Manager: Does the patient understand all the causes of schizophrenia?

- Yes
- No [P1]
- Unknown [P1]

Q2. Case Manager: Does the patient understand all the symptoms of schizophrenia?

- Yes
- No [P1]
- Unknown [P1]

Q3. Case Manager: Does the patient understand how to care for schizophrenia? **QM**

- Yes
- No [P1] [P2]
- Unknown [P1] [P2]

Q4. Case Manager: Does the patient know when and where to get help for problems?

- Yes
- No [P1] [P3]
- Unknown [P1] [P3]

Q5. Case Manager: Does the patient have the ability to understand and follow the self-care plan? **QM**

- Yes
- No [P3]
- Unknown [P3]

Q6. Case Manager: Does the patient understand the purpose and side effects of their schizophrenia medication?

- Yes
- No [P5]
- Unknown [P5]

Q7. Case Manager: Is the patient having trouble coping with their schizophrenia? **QM**

- Yes [P3]
- No
- Unknown [P3]

Q8. Case Manager: Does the patient agree to education? **QM**

- Yes
- No, explain why: _____ [P3] Readiness to Change  CCG
- Unknown [P3]

Quality Measures

- Disease Management Quality Measures:
 - Information provided from this question may be requested by various accrediting organizations.
- HEDIS Measures include(3):

- o Tobacco use is assessed and addressed.
- o Readmission risk is assessed and addressed.
- o Alcohol and substance misuse is assessed and addressed.

Plan of Care

Problems

P1. Schizophrenia information needed

- **Goal:** Schizophrenia diagnosis better understood(4)(5) 
- o **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling
 - **EDUCATE: Definition(6)(7):**
 - Complex brain disease that results in impaired thoughts, perceptions, cognitive functions, moods, and motivations
 - Often called a psychotic disorder because individuals lose touch with reality and exhibit psychotic symptoms (eg, hallucinations, delusions, or catatonia)
 - Causes profound disruption in lives of those diagnosed and the people around them 
 - Lifelong debilitating illness with no cure
 - **EDUCATE: Causes(6)(7):**
 - Exact cause is unknown.
 - Family history (genetics)
 - Traumatic event history
 - Biochemical influences (eg, natural levels of dopamine, serotonin, and norepinephrine)
 - Brain structure and cellular changes
 - **EDUCATE: Symptoms(8):**
 - Cognitive changes (eg, confusion, memory loss)
 - Delusions (eg, paranoid)
 - Emotional affect alteration (eg, flat, inappropriate, blunt, labile)
 - Social changes (eg, isolation, withdrawal)
 - Agitation
 - Hallucinations
 - Thought process problems (eg, disorganized speech, trouble completing activities, concentration difficulty)
 - Sleep problems
 - **EDUCATE: Diagnosis(9)(10):**
 - Based on medical history, risk factors, physical examination, and diagnostic tests
 - Behavioral health examination and history
 - **EDUCATE: Treatment:**
 - ADL and IADL support
 - Behavioral health therapy options
 - Comorbid condition management(11)
 - Medication management
 - No cure
 - Psychotherapy (eg, cognitive behavioral therapy (CBT), individual or group therapy, family therapy)
 - Self-care activities (eg, diet, activity, medication)
 - Symptom management(12)
 - **EDUCATE: Treatment goals:**
 - Complication and progression management
 - Educate and support caregivers
 - Psychosocial functioning improved and maintained
 - Quality of life improvement
 - Recurrence prevention
 - Relapse reduction(12)(13)
 - Remission(10)
 - Safety (for self and others)
 - Symptom frequency and severity reduced
 - Functional improvement and maintenance (eg, ADL, IADL)
 - **EDUCATE: Resources:**
 - National Alliance on Mental Illness: www.nami.org
 - National Institute of Mental Health: www.nimh.nih.gov
 - National Suicide Prevention Lifeline: www.988lifeline.org

- Smokefree. National Institutes of Health: www.smokefree.gov
- **COORDINATE:** Behavioral health services referral, if indicated
- **COORDINATE:** Psychiatrist referral, if indicated(7)
- **COORDINATE:** Laboratory testing, if indicated
- **COORDINATE:** Social worker referral, as needed(14)
- **COORDINATE:** Home care referral, if indicated(6)
- **COORDINATE:** Care team meeting, if needed(15)(16)
- **COORDINATE:** Communication between care team members and care plan reconciliation(15)(17)(18)
- **COORDINATE:** Follow-up to evaluate schizophrenia understanding, referral status, and need for further assistance(19)(20)
- **SEND:** Your care plan - Schizophrenia  (Spanish)
- **SEND:** What you need to know - Schizophrenia  (Spanish)
- **SEND:** When to get help - Schizophrenia  (Spanish)
- **SEND:** Daily care for your condition - Schizophrenia  (Spanish)
- **SEND:** Be ready for your doctor visit - Schizophrenia  (Spanish)
- **SEND:** Be ready with questions for your doctor - Schizophrenia  (Spanish)
- **SEND:** Tips for good mental health  (Spanish)

P2. Schizophrenia self-care needed

- **Goal:** Schizophrenia self-care plan developed, implemented, and maintained(4)(21)(22)[N](#)
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Support system (eg, family, friends) identification, enlistment, and education
 - **ASSIST:** Medical alert bracelet/necklace, obtain and wear
 - **ASSIST:** Appointment scheduling
 - **ASSIST:** Self-care plan creation between patient and provider (eg, goals, management, treating symptoms, getting help)
 - **ASSIST:** Communication between multiple providers(23)
 - **EDUCATE: Self-care plan information(12):**
 - Action plan (eg, when to initiate relaxation, cognitive, and stress-management techniques)
 - Adherence to plan necessary to achieve goals, and complications of nonadherence (eg, exacerbations, progression, comorbidities)[C](#)(8)
 - Auditory hallucination care techniques (eg, use headphones with music, talk to others)
 - Behavioral health treatment (eg, cognitive behavioral therapy (CBT), individual therapy, family therapy)[D](#)(6)(9)[N](#)[N](#)
 - Caregiver support plan, as needed
 - Crisis care plan: turn off distractions (eg, TV, radio), seek emergent care, call hotline, call police if escalates; caregivers advised to not argue(6)(24)
 - Daily care (eg, keep a routine, good hygiene, take medicine)
 - Diet considerations (eg, limit caffeine, avoid alcohol)(9)(25)
 - Exercise program within provider's recommendations(26)[N](#)[N](#)
 - Functional changes care (eg, adaptations and support for ADL and IADL)
 - Health checks, diary, and action plans (eg, write down thoughts, feelings, daily activities, eating times, and when medication taken)(6)
 - Medication purpose, use, effects, and considerations (eg, timing before meals or exercise)(27)
 - Relapse signs (eg, insomnia, decreased appetite, poor personal hygiene)(8)
 - Safety awareness and management (eg, remove suicide risks, de-escalation techniques)
 - Self-care plan guided adjustment, if indicated
 - Set small goals every day (eg, get out of bed at a certain time, finish one chore or project, or do something that brings pleasure like listen to music).(6)(12)
 - Sleep hygiene (eg, regular pattern, calm sleep environment)
 - Substance use avoidance[E](#)(6)(9)[N](#)
 - Support group participation
 - Tobacco cessation plan(27)[N](#)[N](#)
 - Trigger avoidance (eg, high-stress situations, sleep deprivation)(8)
 - Wellness checkups (eg, vaccinations, screenings)
 - **EDUCATE:** Caregiver interventions to support patient self-care(28):
 - Avoid confrontation unless necessary to prevent harmful behavior.(7)(8)

- Delusion and hallucination management techniques (eg, do not disagree or argue, maintain reality, use distractions, ask about hallucination)(6)
- Directions should be specific and concrete; avoid relying on nonverbal communication.
- Learn about disorder, symptoms, precipitating factors, and comorbidities.
- Orient, support, and assist patient as needed.(8)
- Problem-solving techniques (eg, negotiation)(11)
- Promote personal self-care as much as possible; praise successes.(8)
- Set limits; provide structure and safe environment.
- **EDUCATE: When to get emergency help(29):**
 - Suicidal or homicidal actions or plans(8)
 - Heart attack (eg, persistent chest pain; pain radiating to the arms, shoulders, neck, back, and/or jaw; cool clammy skin)(30)
 - Stroke (eg, face drooping, arm weakness, speech problems, bad headache, vision changes, loss of balance or coordination)(31)
 - Crisis plan not working
 - Psychotic behavior (eg, threatening)
 - Respiratory impairment (eg, gasping for air, choking, cyanosis, chest pain, severe dyspnea)(32)
 - Fatigue, extreme
 - Hallucinations or delusions, increased or new
 - Delirium (eg, reduced awareness, agitation)
 - Seizures, new or complicated (eg, seizure longer than 5 minutes, multiple seizures with loss of consciousness, head injury during seizure)(33)
 - Bowel or bladder function loss
- **EDUCATE: When to call the provider for behavior changes(8)(12):**
 - Appointments missed (eg, support group, provider)
 - Cognitive changes (eg, thinking problems, forgetfulness worsens)
 - Disorganization of thoughts (eg, flight of ideas, rapidly switching subjects in conversation)
 - Hallucinations or delusions increase.
 - Mood changes (eg, depression, anger, agitation)
 - Self-care changes (eg, ADL or IADL problems)
 - Self-harm (eg, abuse, mutilation)
 - Social isolation
 - Substance misuse (eg, increased use, use interferes with ADL/IADL)
 - Suspiciousness or paranoia increase
 - Life event, major (eg, job loss, death of someone close)
- **EDUCATE: When to call the provider(8)(12):**
 - Weight gain or loss without reason
 - Constipation
 - Medication problems (eg, side effects, access, administration)
 - Medications newly ordered from other providers
 - Pregnant or planning to get pregnant
- **COORDINATE:** Behavioral health services referral, if indicated(6)
- **COORDINATE:** Psychiatrist referral, if indicated(7)
- **COORDINATE:** Community services referral, as needed(34)
- **COORDINATE:** Crisis services referral, as needed
- **COORDINATE:** Home care referral, if indicated
- **COORDINATE:** Telehealth referral, if indicated[F]
- **COORDINATE:** Social worker referral, as needed(14)
- **COORDINATE:** Support group referral, as needed(34)
- **COORDINATE:** Treatment program referral, if indicated(6)
- **COORDINATE:** Tobacco use treatment, as needed
- **COORDINATE:** Vocational rehabilitation referral, as needed(6)(27)
- **COORDINATE:** Care team meeting, if needed(15)(16)
- **COORDINATE:** Communication between care team members and care plan reconciliation(15)(17)(18)
- **COORDINATE:** Follow-up contact with patient to evaluate understanding and adherence to self-care plan, referral status, and need for further assistance(19)(20)
- **SEND:** Your care plan - Schizophrenia  (Spanish)
- **SEND:** What you need to know - Schizophrenia  (Spanish)
- **SEND:** When to get help - Schizophrenia  (Spanish)
- **SEND:** Daily care for your condition - Schizophrenia  (Spanish)
- **SEND:** Be ready for your doctor visit - Schizophrenia  (Spanish)

- **SEND:** Be ready with questions for your doctor - Schizophrenia  (Spanish)
- **SEND:** Caregiver tips  (Spanish)
- **SEND:** Choosing a place to live  (Spanish)
- **SEND:** Help with sleeping or insomnia  (Spanish)
- **SEND:** How to stop smoking  (Spanish)
- **SEND:** Tips for good mental health  (Spanish)
- **SEND:** Pneumonia vaccine for adults  (Spanish)
- **SEND:** Flu vaccine  (Spanish)
- **SEND:** Symptoms diary (Illustrated)  (Spanish)

P3. Schizophrenia potential complications and comorbidities monitoring needed

- **Goal:** Schizophrenia potential complication or comorbidity evaluated and addressed(4)
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling for evaluation of problems, comorbidities, and self-care plan[G]:
 - Recently hospitalized patient's evaluation within 1 week of discharge
 - Recently hospitalized patient's evaluation with outpatient healthcare providers (eg, nurse, social worker) within 1 to 3 days of discharge(19)
 - Appointment every 3 to 6 months when patient is symptom free
 - Appointment once a month or more for patient with questionable adherence to treatment plan
 - **EDUCATE:** When and how to get help for worsening schizophrenia problems
 - **EDUCATE:** Treatment and monitoring for comorbid conditions (eg, coronary artery disease, high blood pressure, heart valve disease)(12)
 - **EDUCATE:** Follow-up appointments for assessment of clinical status
 - **EDUCATE:** Certain medications prescribed for comorbid conditions may trigger or worsen schizophrenia.(27)(35)
 - **EDUCATE:** Effects of behavioral health condition and conjunctive medication usage during pregnancy and lactation(36)(37)
 - **COORDINATE:** Specialist referral, if indicated(28)
 - **COORDINATE:** Home care referral, if indicated
 - **COORDINATE:** Preconception specialist referral, as needed(36)(37)
 - **COORDINATE:** Treatment program referral, if indicated(34)
 - **COORDINATE:** Vocational rehabilitation referral, as needed(6)(27)
 - **COORDINATE:** Care team meeting, if needed(15)(16)
 - **COORDINATE:** Communication between care team members and care plan reconciliation(15)(17)(18)
 - **COORDINATE:** Follow-up contact to review self-care plan and need for further assistance(19)(20)
 - **SEND:** Be ready for your doctor visit - Substance-related disorders  (Spanish)
 - **SEND:** Be ready with questions for your doctor - Substance-related disorders  (Spanish)
 - **SEND:** Narcotic medicine side effects  (Spanish)
 - **SEND:** Pain medicines - Addiction and withdrawal  (Spanish)

P4. Schizophrenia medication evaluation needed

- **Goal:** Schizophrenia medication evaluation completed and medication regimen initiated, if indicated
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling with provider for evaluation of condition status
 - **EDUCATE:** When and how to get help for worsening schizophrenia problems
 - **EDUCATE:** Medication management needs common for schizophrenia
 - **COORDINATE:** Follow-up after provider contact for evaluation of medication changes(19)(20)

P5. Schizophrenia medication management needed

- **Goal:** Medication information understood(38)
 - **Interventions:**

- **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
- **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
- **ASSIST:** Appointment scheduling with provider for discussion on appropriate medication
- **ASSIST:** Providing reconciled medication list to all providers. See Medication Reconciliation Tool  SR for more information.
- **EDUCATE: Purpose:** Importance of understanding what each medication is for, how it should be taken, and what to do if dose is missed
- **EDUCATE:** Medication indications, side effects, and considerations frequently change. See a drug reference for specific information not listed here.
- **EDUCATE:** Administration techniques:
 - Route (eg, oral, intramuscular (IM), intravenous (IV))
 - With or without foods or liquids (eg, milk)
 - Timing of administration
 - Appropriate measuring device (eg, liquid medications)
 - Proper storage
- **EDUCATE: Get emergency help:** Adverse or allergic reactions (eg, wheezing, chest tightness, fever, itching, bad cough, blue skin color, hemorrhage, seizures, swelling of face, lips, tongue, or throat)
- **EDUCATE: Side effects:** Management, and when to contact provider
 - Common side effects include gastrointestinal changes, dry mouth, fatigue, insomnia, rash, dizziness, and headache.
- **EDUCATE: Considerations:**
 - Discuss medication with provider if pregnant, planning a pregnancy, or breast-feeding.
 - Comorbidities may be contraindications to the use of a drug (eg, liver disease, diabetes).
 - Health checks, diary, and action plans (eg, blood pressure, weight, heart rate)
 - Safety measures (eg, sun protection, driving, or operating machinery)
 - Contact provider or pharmacist prior to taking any other over-the-counter or prescribed medication.
- **COORDINATE:** Clinical monitoring appropriate to medication use (eg, laboratory tests)
- **COORDINATE:** Communication between patient and pharmacy (for obtaining prescriptions, setting up home medication delivery, review for drug-drug interactions) 
- **COORDINATE:** Communication between care team members and medication list reconciliation
- **COORDINATE:** Follow-up to evaluate adherence or problems with medication, provide additional education and support, and identify need for further assistance(19)(20)
- **COORDINATE:** Laboratory testing, if indicated
- **COORDINATE:** Outpatient imaging, tests, or procedures, if indicated
- **COORDINATE:** Therapeutic drug level monitoring
- **SEND:** Managing your medicines (Illustrated)  (Spanish)

P6. Atypical antipsychotic medication management needed

- **Goal:** Atypical antipsychotic medication initiated and maintained
 - **Interventions:**
 - **ASSIST:** Appointment scheduling with prescribing provider for medication evaluation and refills
 - **EDUCATE: Purpose:** Atypical antipsychotics may be indicated for rapid control of acute mania to decrease agitation, aggression, and dangerous behavior.(39)(40)
 - Second-generation antipsychotic medication
 - **EDUCATE: Call provider immediately:** Neuroleptic malignant syndrome (NMS) (eg, extreme muscle rigidity, high fever, tachycardia, tachypnea, diaphoresis, stupor, fluctuations in blood pressure)[H](41)
 - **EDUCATE: Side effects:** Difficulty urinating; jaundice; shuffling walk; fainting spells; seizures; pharyngitis; muscle spasms; excitement or restlessness; jerky movements of head, face, or neck; tachycardia; weight gain; blurred vision(42)
 - Increased risk of metabolic side effects, including obesity, diabetes, and dyslipidemia(40)
 - **EDUCATE: Considerations(40):**
 - Use cautiously with history of seizures, liver disease, heart disease, blood disorders, or pregnancy.(43)
 - Monitor blood glucose, cholesterol, and triglyceride levels.
 - Talk to provider before discontinuing medication.
 - **COORDINATE:** Electrocardiogram (ECG) monitoring, if indicated(40)
 - **COORDINATE:** Laboratory testing

P7. Typical antipsychotic medication management needed

- **Goal:** Typical antipsychotic medication initiated and maintained(40)
 - **Interventions:**

- **EDUCATE: Purpose:** Antipsychotics are used to treat hallucinations, delusions, disorganized thoughts and behavior, and related agitation/aggression.
 - Long-acting injectable (LAI) antipsychotics may improve adherence.
- **EDUCATE: Call provider immediately:** Neuroleptic malignant syndrome (NMS) (eg, extreme muscle rigidity, high fever, tachycardia, tachypnea, diaphoresis, stupor, fluctuations in blood pressure)[H](41)
- **EDUCATE: Side effects:** Drowsiness, dry mouth, weight gain, ECG changes, involuntary reflexes and movements, hyperglycemia, diabetes
 - Tardive dyskinesia (involuntary repetitive movements, such as grimacing, tongue movements, lip smacking, pursed lips, excessive eye blinking) can occur.[I]
 - Increased risk of metabolic side effects, including obesity, diabetes, and dyslipidemia
- **EDUCATE: Considerations:**
 - PRN medications may be ordered to address involuntary reflexes and movements.
 - Monitor blood glucose, cholesterol, and triglyceride levels.
 - Diet and exercise instruction
 - Modest efficacy and increases risk of cognitive decline and cerebrovascular events[J](43)
 - Talk to provider before discontinuing medication.
- **COORDINATE:** Electrocardiogram (ECG) monitoring, if indicated

P8. Sleeping problems present

- **Goal:** Sleep improved[N]
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Scheduling provider appointment for medical evaluation of sleep changes and creation of self-care plan to include(44)(45):
 - Identifying techniques to improve sleep quality(46)
 - Medication changes that can help patient sleep better (eg, changing the time stimulants are taken to early in the day or not taking diuretic before bedtime)
 - Review of herbal preparations that may promote sleep(47)
 - Creation of sleep diary(48)
 - **EDUCATE:** Sleep hygiene techniques(44)(46)(49):
 - Schedule sleep (go to bed and get up at same time every day).
 - Spend no more than 8 hours per day in bed. Amount of average sleep time for adult is 7 to 8 hours.
 - Limit napping to once a day for 20 to 30 minutes in early afternoon.
 - Sleep in a dark, quiet bedroom.
 - Use bedroom only for sleeping, not to watch TV, work, use electronics, or do other activities.
 - Avoid pets in the bed.
 - Get safe amount of sunlight every day and avoid bright lights before bedtime.
 - Cut down on drinking fluids in the afternoon if nighttime urination is a problem.
 - Avoid substances that may influence sleep (eg, caffeine, stimulants, alcohol).
 - Stop smoking; nicotine is a stimulant.
 - Get regular exercise, but avoid vigorous exercise 2 hours before bedtime.(50)
 - Write down urgent thoughts if mind-racing is a problem.
 - Do not eat big meals before bedtime; gastric process can cause sleep problems.
 - **EDUCATE:** Herbal preparations to help sleep (eg, melatonin, tryptophan, valerian), as approved by provider(51)[N]
 - **EDUCATE:** Behavioral health treatment options and benefits (eg, cognitive behavioral therapy for insomnia)(48)(52)[N]
 - **EDUCATE:** Hypnotic medications use and misuse, if prescribed for insomnia(45)
 - **COORDINATE:** Sleep specialist referral, if indicated(51)
 - **COORDINATE:** Behavioral health services referral, if indicated(48)
 - **COORDINATE:** Communication between care team members and care plan reconciliation(15)(17)(18)
 - **COORDINATE:** Follow-up contact to evaluate sleep, status of referrals, and need for further assistance(19)(20)
 - **SEND:** Help with sleeping or insomnia  (Spanish)

P9. Home safety management information needed

- **Goal:** Home safety management plan understood and followed(53)
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)

- **ASSIST:** Appointment scheduling for evaluation of dizziness, unexplained falls, memory loss, medication issues, or other home safety concerns^[N]
- **ASSIST:** Identifying patient's needs, abilities, preferences, and risks for problems. See Home Safety Assessment [SR](#) for further information.^{[K](54)}
- **ASSIST:** Medication list and pharmacy review⁽⁴⁴⁾
- **ASSIST:** Identifying environmental structural problems or modifications (eg, steps, lighting, access to bathroom or kitchen)⁽⁵⁵⁾
- **ASSIST:** Obtaining emergency medical identification, if needed
- **ASSIST:** Acquiring and using devices for communication (eg, cell phone, alarm system) in case of emergency⁽⁵⁶⁾
- **ASSIST:** Identifying and moving to different living arrangements, as needed
- **ASSIST:** Creating emergency evacuation plan, as needed
- **EDUCATE:** Home changes to help prevent falls:
 - Remove all throw rugs and clutter from floors.
 - Repair carpet tears, broken tiles, or hardwood floor ruts that could cause tripping.
 - Clear walking areas of electrical cords or wires.
 - Move furniture to allow for use of crutches or walker.
 - Keep all areas well lit.⁽⁵⁷⁾
 - Clean up spills immediately.
- **EDUCATE:** Self-care safety activities⁽⁵⁸⁾⁽⁵⁹⁾:
 - Rise slowly from lying or sitting position.
 - Wear glasses or contact lenses, if needed.
 - Use handrails, grab bars, or adaptive devices as needed.
 - Wear shoes with nonslip soles.
 - Use no-slip mats in shower or tub.
 - Pet-related falls mitigation (eg, not chasing pet, obedience training)
 - Know which medication may cause problems (eg, dizziness, weakness, or balance problems).
 - Exercise program within provider's recommendations⁽²⁶⁾⁽⁵⁵⁾^[N]
- **EDUCATE:** Safety hazard removal and ways to cut down safety risks⁽⁵⁹⁾:
 - Architectural changes that can improve safety (eg, widening doors to allow for wheelchair, putting grab bars in bathroom)
 - Hot water tank temperature setting between 110 to 120 degrees F (43.4 to 48.9 degrees C)⁽⁶⁰⁾
 - Snow and ice removal plan
 - Chairs and other seats stable and at height to facilitate easy rising
 - Electrical appliances, cords, and outlets safe, used appropriately, and in good repair
- **EDUCATE:** Fire prevention:
 - Smoke detector functioning, and plan to change battery regularly
 - Fireplace in good condition, with screen and chimney cleaned regularly
 - Smoking habits as safe as possible
 - Electrical and heating units away from flammable material
 - Electrical appliances, cords, and outlets safe, used appropriately, and in good repair
 - Evacuation plan in event of fire that includes location of available exits
 - How to put out flames if clothes catch on fire
- **EDUCATE:** When to get emergency help:
 - Fall complications (eg, head injury, broken bone, significant ADL changes)
 - Fear of personal harm from someone (eg, spouse, parent). See Intimate Partner Violence [CCG](#).
 - Stroke, heart attack, or blood clot
- **EDUCATE:** When to call the provider:
 - Cognitive changes (eg, thinking problems, forgetfulness worsens)
 - Depression (eg, feeling sad, hopeless, overwhelmed)⁽²⁹⁾
 - Falls
 - Medication problems (eg, side effects, access, administration)
 - Self-care changes (eg, ADL or IADL problems)
 - Physical activity level declines (eg, fatigue, weakness).
- **COORDINATE:** Pharmacy medication support, if indicated^{[L](61)}
- **COORDINATE:** Specialist referral, if indicated
- **COORDINATE:** Home care referral, if indicated⁽⁶²⁾
- **COORDINATE:** Rehabilitation therapy referral, if indicated⁽⁵⁵⁾⁽⁶²⁾
- **COORDINATE:** Medical equipment and supplies, if indicated⁽⁶³⁾
- **COORDINATE:** Community services referral, as needed^[M]
- **COORDINATE:** Social worker referral, as needed⁽¹⁴⁾
- **COORDINATE:** Protective services or legal authorities, as needed⁽⁶¹⁾
- **COORDINATE:** Emergency plans for regional weather or other emergencies (eg, earthquakes, floods, tornadoes)

- **COORDINATE:** Follow-up contact to evaluate safety, referral status, and need for further assistance(19)(20)
- **SEND:** Choosing a place to live  (Spanish)
- **SEND:** Exercising to improve balance (Illustrated)  (Spanish)
- **SEND:** Exercising when you have neurologic problems  (Spanish)
- **SEND:** How to get out of bed safely (Illustrated)  (Spanish)
- **SEND:** How to move safely in bed (Illustrated)  (Spanish)
- **SEND:** How to use a cane (Illustrated)  (Spanish)
- **SEND:** How to use a walker (Illustrated)  (Spanish)
- **SEND:** Managing your medicines (Illustrated)  (Spanish)
- **SEND:** Safe lifting (Illustrated)  (Spanish)
- **SEND:** Safety in the home - Fall protection  (Spanish)
- **SEND:** Safety in the home - Fire and other risks  (Spanish)
- **SEND:** Safety in the home - Memory loss  (Spanish)
- **SEND:** Walking with crutches (Illustrated)  (Spanish)
- **SEND:** Ways to save energy  (Spanish)

P10. Home safety issues not managed

- **Goal:** Safety issues identified and addressed(64)
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling for evaluation of dizziness, unexplained falls, memory loss, medication issues, or other home safety concerns
 - **ASSIST:** Contact with legal authorities, as appropriate
 - **EDUCATE:** When and how to get help for worsening problems or concerns for safety
 - **COORDINATE:** Social worker referral, as needed(14)
 - **COORDINATE:** Medical equipment and supplies, if indicated(63)
 - **COORDINATE:** Protective services or legal authorities, as needed
 - **COORDINATE:** Pharmacy medication support, if indicated(61)
 - **COORDINATE:** Rehabilitation therapy referral, if indicated(55)(62)
 - **COORDINATE:** Specialist referral for comorbidities, if indicated
 - **COORDINATE:** Audiologist referral, if indicated
 - **COORDINATE:** Ophthalmologist referral, if indicated
 - **COORDINATE:** Home care referral, if indicated(62)
 - **COORDINATE:** Follow-up contact to evaluate safety issues, referral status, and need for further assistance(19)(20)

P11. Healthy diet information needed

- **Goal:** Healthy diet better understood(65)(66)
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling with provider for dietary review and self-care plan
 - **EDUCATE:** Healthy body weight and body mass index (BMI) for patient(67):
 - BMI equals body weight in kilograms divided by the square of the height in meters (kg/m²).
 - BMI of 18.5 to 24.9 is a normal weight.
 - BMI of 25 to 29.9 is overweight.
 - BMI of 30 or greater is obese.
 - **EDUCATE:** Healthy diet general recommendations(68)(69):
 - Well-balanced diet (ie, high in fruits and vegetables, low in sodium, fats, and sugars)
 - Drink enough fluids.
 - Safe eating habits (eg, do not skip meals)
 - Calorie amount appropriate (eg, factors: age, gender, activity level)
 - **EDUCATE:** Adequate fluid intake^[N](69)(70):

- Amount of fluid intake needed depends on patient attributes, including medical condition, level of physical activity, and exposure to heat.^N
- Men age 19 years and older: 15.6 cups (3.7 L) per day from all sources (eg, food and fluids)
- Women age 19 years and older: 11.4 cups (2.7 L) per day from all sources (eg, food and fluids)
- **COORDINATE:** Dietitian referral, if indicated(71)
- **COORDINATE:** Community services referral, as needed
- **COORDINATE:** Follow-up contact with patient to evaluate diet understanding and adherence, referral status, and need for further assistance(19)(20)
- **SEND:** Healthy eating (Illustrated)  (Spanish)
- **SEND:** How to read food labels (Illustrated)  (Spanish)
- **SEND:** Weight diary (Illustrated)  (Spanish)
- **SEND:** Meal diary (Illustrated)  (Spanish)

P12. Potential nutritional problems

- **Goal:** Nutritional problems resolved(66)(72)
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling with provider for dietary review and self-care plan
 - **ASSIST:** Creation of nutrition diary
 - **COORDINATE:** Laboratory testing, if indicated
 - **COORDINATE:** Calorie or fluid counting
 - **EDUCATE:** Nutritional recommendations for patient's health status(69)
 - **EDUCATE:** Food and drug interactions potentially impacting nutritional status
 - **COORDINATE:** Dietitian referral, if indicated(71)
 - **COORDINATE:** Home care referral, if indicated
 - **COORDINATE:** Pharmacy medication support, if indicated
 - **COORDINATE:** Rehabilitation therapy referral, if indicated
 - **COORDINATE:** Follow-up contact with patient to evaluate nutritional status, referral status, and need for further assistance(19)(20)
 - **SEND:** Eating to keep up your energy  (Spanish)
 - **SEND:** Eating when your stomach is upset  (Spanish)
 - **SEND:** Mouth sores or infections  (Spanish)
 - **SEND:** How to read food labels (Illustrated)  (Spanish)
 - **SEND:** Meal diary (Illustrated)  (Spanish)

P13. Food label reading information needed

- **Goal:** Food label understanding improved
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling with provider, nurse, or dietitian to provide in-person instruction on how to read food labels
 - **EDUCATE:** Food labels contain the nutritional value of the product(73)(74):
 - Percent of daily values is usually based on a 2000-calorie (8374-kJ) diet.
 - Nutritional values are listed per one serving.
 - A package may have more than one serving in the whole package.
 - Nutritional value should be doubled, tripled, and so on based on the number of servings eaten.
 - Serving size varies from product to product.
 - Form of food has impact on nutrition (eg, increased sugar content of orange juice vs one orange).
 - **COORDINATE:** Dietitian referral, if indicated
 - **COORDINATE:** Follow-up contact to evaluate food label understanding, referral status, and need for further assistance(19)(20)
 - **SEND:** How to read food labels (Illustrated)  (Spanish)

P14. Vaccinations not up to date, increasing infection risk

- **Goal:** Vaccinations up to date(75)(76)
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling
 - **ASSIST:** Identifying vaccines patient may need that are not contraindicated based on clinical status
 - **EDUCATE:** Purpose and benefits of vaccinations
 - **EDUCATE:** Recommended immunizations and timing of administration:
 - Centers for Disease Control and Prevention's vaccination schedule: www.cdc.gov/vaccines(77)
 - Vaccine schedules may need to be altered based on medications; check with the specialist provider for information.(78)
 - Possible side effects or complications associated with getting vaccinated
 - **EDUCATE:** Influenza vaccination annually(79):
 - Side effects are mild and last 1 or 2 days.
 - Common side effects with shot are sore, red, or swollen area where shot was given; fever; and aches.
 - Common side effects with nasal spray are mild and last 1 or 2 days; they include runny nose, headache, sore throat, and cough.
 - Allergic reaction rare and is usually caused by a chicken egg allergy
 - **EDUCATE:** Pneumococcal vaccination potential side effects:
 - Side effects are mild and last 1 or 2 days.
 - Common side effects include a sore, red, or swollen area where shot was given; fever; and muscle aches.
 - Serious side effects are rare.
 - **EDUCATE:** COVID-19 vaccination potential side effects(77):
 - Side effects are mild and last 1 or 2 days.
 - Common side effects include a sore, red, or swollen area where shot was given; chills; fatigue; and aches.
 - Serious side effects are rare.
 - **COORDINATE:** Community services referral, as needed
 - **COORDINATE:** Vaccinations
 - **COORDINATE:** Follow-up to evaluate vaccination understanding, referral status, and need for further assistance(19)(20)
 - **SEND:** Coronavirus disease 2019 (COVID-19) vaccines  (Spanish)
 - **SEND:** Flu vaccine  (Spanish)
 - **SEND:** Pneumonia vaccine for adults  (Spanish)
 - **SEND:** Hepatitis A vaccine  (Spanish)
 - **SEND:** Hepatitis B vaccine  (Spanish)

Evidence Summary

Quality standards: The Case Management Society of America's Standards of Practice for Case Management principles include facilitation of informed decision making, counseling, and health education with patient self-care that is responsive to the patient's preferences (eg, culture, values, needs).(80)

Evidence: A systematic review and network meta-analysis found that family interventions, family psychoeducation, and cognitive behavioral therapy were the most beneficial psychological and psychosocial interventions in reducing the risk of relapse for patients with schizophrenia and that clinicians should consider prioritizing these interventions for relapse prevention and long-term treatment.(13)

Evidence: A meta-analysis reported that only 13.5% of patients met strict criteria for recovery, which included the presence of at most mild symptoms and good social functioning, with at least one of these outcomes lasting at least 2 years.(9)

Quality standards: Standards indicate that program content should include an individualized self-care plan that includes patient preferences, prioritized goals, interventions with information on whom to contact in urgent situations, and scheduled follow-up. Programs should facilitate communication and collaboration with practitioners in a timely manner.(80)(81)(82)(83)

National guidelines: The National Institute for Health and Care Excellence (NICE) guidelines suggest that all patients (adults and children) with psychosis or schizophrenia, including those with acute or recurrent symptoms, receive family therapy (if applicable) as an adjunct to individual cognitive behavioral therapy (CBT).(27)

Evidence: Randomized controlled trials and meta-analyses have found that when psychosocial care (eg, family-based intervention) is integrated with standard pharmacologic care, there is a reduced risk of relapse and hospitalization.(27)(84)

Evidence: A review of 24 studies involving 1900 participants suggests that group cognitive behavioral therapy (CBT) may be better than standard care or other psychosocial interventions for overall mental state and global functioning for people with schizophrenia.(85)

National guidelines: The 2018 Physical Activity Guidelines Advisory Committee advises that adults with chronic conditions or disabilities, who are able, should follow the key activity guidelines for adults. The guidelines recommend that adults should do at least 150 to 300 minutes a week of moderate-intensity or 75 to 150 minutes a week of vigorous-intensity aerobic physical activity, or an equivalent combination of moderate-intensity and vigorous-intensity aerobic activity. They should also do muscle-strengthening activities on 2 or more days a week. Older adults should do multicomponent physical activity that includes balance training as well as aerobic and muscle-strengthening activities.(86)

Specialty society: The National Institute for Health and Care Excellence (NICE) recommends offering combined healthy eating and physical activity programs to help reduce the increased rates of obesity and type 2 diabetes found in adults with psychosis or schizophrenia.(11)

Evidence: Studies suggest that having ever used cannabis was associated with a modest increased risk of psychotic disorders, daily use was associated with a higher risk, and daily use of high-potency cannabis conferred a risk of more than 4 times that of ever having used cannabis.(9)

Specialty society: The American Psychiatric Association states that smoking is a major contributor to increased mortality in individuals with serious mental illness and advises that health education and motivational interviewing may be helpful in those who have had prior unsuccessful attempts at smoking cessation.(87)

Specialty society: Smoking tobacco or marijuana reduces the serum levels of several antipsychotic drugs, including clozapine and olanzapine.(87)

Quality standards: Standards indicate that programs should ensure that during care transitions the care plan is re-evaluated, modified, and communicated to appropriate care team members (eg, providers, caregivers) as needed to support the care transition.(80)(81)(82)(83)

Transition consideration: Several studies demonstrated a reduction in 30-day hospital readmissions when the patient follow-up was within 7 days of hospital discharge.(88)(89)(90) Earlier follow-up (eg, within 48 hours after hospital discharge) may also help reduce hospital readmissions for high-risk or vulnerable patients.(19) Follow-up appointment attendance rates were higher when appointments were scheduled before hospital discharge.(91)(92)

National guidelines: The National Institute for Health and Care Excellence (NICE) guidelines recommend the administration of antipsychotics to treat both acute and recurrent symptoms in schizophrenia.(27)

Outcomes: Several studies support the role of the pharmacist in medication reconciliation to identify and resolve pharmacy-related problems and improve outcomes (eg, prevention of medication-related adverse events, discontinuation of inappropriate medication).(93)

Quality standards: Standards indicate that program content should include a condition treatment plan, self-management and self-monitoring, and health behaviors (eg, sleep patterns). Programs should have a process for following up with patients on the status of referrals.(80)

Specialty society: In a systematic review of the pharmacologic treatment of chronic insomnia in adults, the American Academy of Sleep Medicine does not recommend diphenhydramine, melatonin, tryptophan, or valerian for treating sleep onset or maintenance.(47)

Specialty society: The American College of Physicians recommends cognitive behavioral therapy for insomnia as the first-line treatment for patients with chronic insomnia (trouble falling or staying asleep for 3 or more nights a week for at least 3 months, with daytime consequences). Cognitive behavioral therapy for insomnia generally includes cognitive therapy with sleep restriction, relaxation training, and stimulus control for 5 to 8 sessions.(94)

Evidence: Multiple studies indicate that the large number and types of medications consumed by the elderly increase the risk of falls and should be evaluated.(44)(64)(95)

National guidelines: The U.S. Preventive Services Task Force recommends exercise-based interventions to help reduce falls for people 65 years and older. Other interventions (eg, behavioral therapy, nutrition therapy, environmental modification, medication changes) may be considered based on the patient's preferences, the benefit-to-harm balance, and the circumstances of prior falls. Vitamin D supplementation is not recommended for fall prevention for older people without a known deficiency or osteoporosis.(55)

National guidelines: The Institute of Medicine does not specify how much total fluid should come from water. Their recommendation is based on the median of total water intake on dietary surveys, which include water from liquids and solids. For healthy people, the

advice is to consume at least 8 glasses (8 ounces (240 mL) each) of water daily; there is little evidence to support this recommendation. (96)

References

- Chiappetta L. Mood disorders: depression. In: Fosbre CD, Varcарolis EM, Chiappetta L, editors. *Varcarolis' Essentials of Psychiatric-Mental Health Nursing*. 5th ed. Elsevier; 2023:207-234. [Context Link 1]
- US Preventive Services Task Force, et al. Screening for Depression and Suicide Risk in Adults: US Preventive Services Task Force Recommendation Statement. *Journal of the American Medical Association* 2023;329(23):2057-2067. DOI: 10.1001/jama.2023.9297. (Reaffirmed 2023 Jul 06) [Context Link 1] View abstract...
- HEDIS quality measures 2023. [Internet] NCQA. Accessed at: <https://www.ncqa.org/hedis/measures/>. Updated 2023 [accessed 2023 Oct 23] [Context Link 1]
- Assessing the adult patient through application of the integrated case management complexity assessment grid. In: Fraser K, Perez R, Latour C, editors. *CMSA's Integrated Case Management*. New York, NY: Springer Publishing Company LLC; 2018:51-82. [Context Link 1, 2, 3]
- Improving adherence. In: Perez R, Rogers S, Prince M, Fraser K, editors. *Case Management Adherence Guidelines*. 2020 ed. Case Management Society of America; 2022:19-42. [Context Link 1]
- Schizophrenia. In: Videbeck SL, editor. *Psychiatric-Mental Health Nursing*. 9th ed. Wolters Kluwer; 2023:254-283. [Context Link 1, 2, 3, 4, 5, 6, 7, 8, 9, 10, 11, 12, 13, 14]
- Chiappetta L. Schizophrenia spectrum disorders and other psychotic disorders. In: Fosbre CD, Varcарolis EM, Chiappetta L, editors. *Varcarolis' Essentials of Psychiatric-Mental Health Nursing*. 5th ed. Elsevier; 2023:257-285. [Context Link 1, 2, 3, 4, 5]
- Keltner NL, Smith-East M. Schizophrenia spectrum and other psychotic disorders. In: Steele D, editor. *Keltner's Psychiatric Nursing*. 9th ed. Elsevier; 2023:256-278. [Context Link 1, 2, 3, 4, 5, 6, 7, 8, 9, 10, 11]
- Jauhar S, Johnstone M, McKenna PJ. Schizophrenia. *Lancet* 2022;399(10323):473-486. DOI: 10.1016/S0140-6736(21)01730-X. [Context Link 1, 2, 3, 4, 5, 6] View abstract...
- Reus VI. Psychiatric disorders. In: Loscalzo J, Fauci A, Kasper D, Hauser S, Longo D, Jameson JL, editors. *Harrison's Principles of Internal Medicine*. 21st ed. McGraw Hill Education; 2022:3540-3556. [Context Link 1, 2]
- Psychosis and Schizophrenia in Adults. NICE Quality Standards QS80 [Internet] National Institute for Health and Care Excellence. 2015 Feb Accessed at: <https://www.nice.org.uk/guidance/>. [accessed 2023 Aug 08] [Context Link 1, 2, 3]
- Schizophrenia spectrum and other psychotic disorders. In: Morgan KI, Townsend MC, editors. *Davis Advantage for Psychiatric Mental Health Nursing*. 10th ed. Philadelphia, PA: F.A. Davis; 2021:441-478. [Context Link 1, 2, 3, 4, 5, 6, 7]
- Bighelli I, et al. Psychosocial and psychological interventions for relapse prevention in schizophrenia: a systematic review and network meta-analysis. *Lancet Psychiatry* 2021;8(11):969-980. DOI: 10.1016/S2215-0366(21)00243-1. [Context Link 1, 2] View abstract...
- McQueen A, et al. Social needs, chronic conditions, and health care utilization among Medicaid beneficiaries. *Population Health Management* 2021;24(6):681-690. DOI: 10.1089/pop.2021.0065. [Context Link 1, 2, 3, 4] View abstract...
- Case management standards and professional organizations. In: Powell SK, Tahan H, editors. *Case Management a Practical Guide for Education and Practice*. 4th ed. Philadelphia, PA: Wolters Kluwer, Lippincott, Williams & Wilkins; 2019:314-354. [Context Link 1, 2, 3, 4, 5, 6, 7]
- Ralston JD, Wagner EH. Comprehensive chronic disease management. In: Goldman L, Schafer AI, editors. *Goldman-Cecil Medicine*. 26th ed. Elsevier; 2020:42-45.e2. [Context Link 1, 2, 3]
- Karam M, et al. Nursing care coordination for patients with complex needs in primary healthcare: a scoping review. *International Journal of Integrated Care* 2021;21(1):16. DOI: 10.5334/ijic.5518. [Context Link 1, 2, 3, 4] View abstract...
- Joo JY, Liu MF. The experience of chronic illness transitional care: a qualitative systematic review. *Clinical Nursing Research* 2022;31(2):163-173. DOI: 10.1177/10547738211056166. [Context Link 1, 2, 3, 4] View abstract...
- Care coordination and care transitions to support adherence. In: Perez R, Rogers S, Prince M, Fraser K, editors. *Case Management Adherence Guidelines*. 2020 ed. Case Management Society of America; 2022:43-56. [Context Link 1, 2, 3, 4, 5, 6, 7, 8, 9, 10, 11, 12, 13, 14]
- Monitoring and evaluation. In: Gillingham DC, editor. *Foundations of Case Management*. Blue Bayou Press; 2021:51-52. [Context Link 1, 2, 3, 4, 5, 6, 7, 8, 9, 10, 11, 12]
- Bennett WL, et al. Strategies for Patient, Family, and Caregiver Engagement. Technical Brief Number 36. AHRQ Publication No. 20-EHC017 [Internet] Agency for Healthcare Research and Quality Effective Health Care Program. 2020 Aug Accessed at: <https://effectivehealthcare.ahrq.gov/>. [accessed 2023 Sep 13] DOI: 10.23970/AHRQEPCTB36. [Context Link 1]
- Aboumatar H, et al. Patient engagement strategies for adults with chronic conditions: an evidence map. *Systematic Reviews* 2022;11(1):Online. DOI: 10.1186/s13643-021-01873-5. [Context Link 1] View abstract...
- Hudon C, et al. Case management in primary care for frequent users of health care services: a realist synthesis. *Annals of Family Medicine* 2020;18(3):218-226. DOI: 10.1370/afm.2499. [Context Link 1] View abstract...
- Fosbre CD. Anger, aggression, and violence. In: Fosbre CD, Varcарolis EM, Chiappetta L, editors. *Varcarolis' Essentials of Psychiatric-Mental Health Nursing*. 5th ed. Elsevier; 2023:388-401. [Context Link 1]
- Neuropsychiatric disorders. In: Escott-Stump S, editor. *Nutrition and Diagnosis-Related Care*. 9th ed. Wolters Kluwer; 2022:276-378. [Context Link 1]
- Physical Activity and Sedentary Behaviour. WHO Guideline [Internet] World Health Organization. 2020 Nov Accessed at: <https://www.who.int/publications/i/item/9789240015128>. [accessed 2023 Aug 14] [Context Link 1, 2]

27. Psychosis and Schizophrenia in Adults: Treatment and Management. NICE Clinical Guidance CG178 [Internet] National Institute for Health and Care Excellence. 2014 Mar (NICE reviewed 2021) Accessed at: <https://www.nice.org.uk/guidance>. [created 2014; accessed 2023 Aug 08] [Context Link 1, 2, 3, 4, 5, 6, 7, 8, 9]
28. Schizophrenia. In: Swearingen PL, Wright JD, editors. All-in-One Nursing Care Planning Resource. 5th ed. St. Louis, MO: Elsevier; 2019:753-758. [Context Link 1, 2]
29. Mood disorders and suicide. In: Videbeck SL, editor. Psychiatric-Mental Health Nursing. 9th ed. Wolters Kluwer; 2023:284-327. [Context Link 1, 2]
30. Shaffer RB. Coronary artery disease and acute coronary syndrome. In: Harding MM, Kwong J, Hagler D, Reinisch C, editors. Lewis's Medical-Surgical Nursing: Assessment and Management of Clinical Problems. 12th ed. St. Louis, MO: Mosby; 2023:819-858. [Context Link 1]
31. Bussard M. Stroke. In: Harding MM, Kwong J, Hagler D, Reinisch C, editors. Lewis's Medical-Surgical Nursing: Assessment and Management of Clinical Problems. 12th ed. St. Louis, MO: Mosby; 2023:1515-1538. [Context Link 1]
32. Mondor EE. Lower respiratory problems. In: Harding MM, Kwong J, Hagler D, Reinisch C, editors. Lewis's Medical-Surgical Nursing: Assessment and Management of Clinical Problems. 12th ed. St. Louis, MO: Mosby; 2023:596-631. [Context Link 1]
33. Neurologic care. In: Lynn P, editor. Taylor's Clinical Nursing Skills. 6th ed. Wolters Kluwer; 2023:1079-1113. [Context Link 1]
34. Cohen DJ, Davis MM, Hall JD, Gilchrist EC, Miller BF. A Guidebook of Professional Practices for Behavioral Health and Primary Care Integration. The Academy Integrating Behavioral Health and Primary Care AHRQ Publication No. 14-0070-1-EF [Internet] Agency for Healthcare Research and Quality. 2015 Mar Accessed at: <https://www.ahrq.gov/>. [accessed 2023 Aug 15] [Context Link 1, 2, 3]
35. Bipolar and related disorders. In: Morgan KI, Townsend MC, editors. Davis Advantage for Psychiatric Mental Health Nursing. 10th ed. Philadelphia, PA: F.A. Davis; 2021:519-546. [Context Link 1]
36. Gonzalez-Rodriguez A, et al. Women with schizophrenia over the life span: health promotion, treatment and outcomes. International Journal of Environmental Research and Public Health 2020;17(15):Online. DOI: 10.3390/ijerph17155594. [Context Link 1, 2] View abstract...
37. Reinstein SA, Cosgrove J, Malekshahi T, Deligiannidis KM. Long-acting injectable antipsychotic use during pregnancy: a brief review and concise guide for clinicians. Journal of Clinical Psychiatry 2020;81(6):20ac13597. DOI: 10.4088/JCP.20ac13597. [Context Link 1, 2] View abstract...
38. The nursing process in drug therapy and patient safety. In: Tucker RG, editor. Karch's Focus on Nursing Pharmacology. 9th ed. Wolters Kluwer; 2023:47-57. [Context Link 1]
39. Drugs for bipolar disorder. In: Burchum JR, Rosenthal LD, editors. Lehne's Pharmacology for Nursing Care. 11th ed. Elsevier; 2022:372-379. [Context Link 1]
40. Psychotherapeutic agents. In: Tucker RG, editor. Karch's Focus on Nursing Pharmacology. 9th ed. Wolters Kluwer; 2023:376-395. [Context Link 1, 2, 3, 4, 5]
41. Neurobiologic theories and psychopharmacology. In: Videbeck SL, editor. Psychiatric-Mental Health Nursing. 9th ed. Wolters Kluwer; 2023:15-40. [Context Link 1, 2, 3]
42. Antipsychotic agents and their use in schizophrenia. In: Burchum JR, Rosenthal LD, editors. Lehne's Pharmacology for Nursing Care. 11th ed. Elsevier; 2022:334-350. [Context Link 1, 2]
43. Psychopharmacology. In: Morgan KI, Townsend MC, editors. Davis Advantage for Psychiatric Mental Health Nursing. 10th ed. Philadelphia, PA: F.A. Davis; 2021:53-86. [Context Link 1, 2, 3]
44. A - W. In: Phelps LL, editor. Nursing Diagnosis Reference Manual. 12th ed. Wolters Kluwer; 2023:2-750. [Context Link 1, 2, 3, 4]
45. Greenough GP, Judd BG. Sleep disorders. In: Buttaro TM, Polgar-Bailey P, Sandberg-Cook J, Trybulski JA, editors. Primary Care: A Collaborative Practice. 6th ed. St. Louis, MO: Elsevier; 2021:1252-1262. [Context Link 1, 2]
46. Gold R. Insomnia. In: Ferri FF, editor. Ferri's Clinical Advisor. 2024 ed. Elsevier; 2024:804.e13-804.e22. [Context Link 1, 2]
47. Sateia MJ, Buysse DJ, Krystal AD, Neubauer DN, Heald JL. Clinical practice guideline for the pharmacologic treatment of chronic insomnia in adults: an American Academy of Sleep Medicine clinical practice guideline. Journal of Clinical Sleep Medicine 2017;13(2):307-349. DOI: 10.5664/jcsm.6470. (Reaffirmed 2023 Jun) [Context Link 1, 2] View abstract...
48. Weilburg JB, Winkelman J. Approach to the patient with insomnia. In: Goroll AH, Mulley AG Jr., editors. Primary Care Medicine: Office Evaluation and Management of the Adult Patient. 8th ed. Philadelphia, PA: Wolters Kluwer; 2021:1760-1771. [Context Link 1, 2, 3]
49. Appendix A: Disorders of sleep and wakefulness. In: Videbeck SL, editor. Psychiatric-Mental Health Nursing. 9th ed. Wolters Kluwer; 2023:483-485. [Context Link 1]
50. Sejbuk M, Mironczuk-Chodakowska I, Witkowska AM. Sleep quality: a narrative review on nutrition, stimulants, and physical activity as important factors. Nutrients 2022;14(9):Online. DOI: 10.3390/nu14091912. [Context Link 1] View abstract...
51. Mansukhani MP, Kolla BP, St. Louis EK. Sleep disorders. In: Kellerman RD, Rakel DP, Heidelbaugh JJ, Lee EM, editors. Conn's Current Therapy. 2023 ed. Philadelphia, PA: Elsevier; 2023:794-809. [Context Link 1, 2]
52. Edinger JD, et al. Behavioral and psychological treatments for chronic insomnia disorder in adults: an American Academy of Sleep Medicine clinical practice guideline. Journal of Clinical Sleep Medicine 2021;17(2):255-262. DOI: 10.5664/jcsm.8986. [Context Link 1] View abstract...
53. Laplante N. Home care safety. In: Perry AG, Potter PA, Ostendorf WR, editors. Nursing Interventions and Clinical Skills. 7th ed. Elsevier; 2020:842-862. [Context Link 1, 2]
54. Wright E, Stanley MJ. Home health and hospice care. In: Rector C, Stanley MJ, editors. Community and Public Health Nursing Promoting the Public's Health. 10th ed. Wolters Kluwer; 2022:850-867. [Context Link 1]
55. US Preventive Services Task Force, et al. Interventions to prevent falls in community-dwelling older adults: US Preventive Services Task Force recommendation statement. Journal of the American Medical Association 2018;319(16):1696-1704. DOI: 10.1001/jama.2018.3097. (Reaffirmed 2023 Feb) [Context Link 1, 2, 3, 4, 5] View abstract...

56. Rudolphi DM. Postoperative care. In: Harding MM, Kwong J, Hagler D, Reinisch C, editors. *Lewis's Medical-Surgical Nursing: Assessment and Management of Clinical Problems*. 12th ed. St. Louis, MO: Mosby; 2023:365-384. [Context Link 1]
57. Care plans for clinical problems. In: Gulanick M, Myers JL, editors. *Nursing Care Plans*. 10th ed. Elsevier; 2022:180-258. [Context Link 1]
58. Nursing care of the elderly client. In: Haugen N, Galura SJ, editors. *Ulrich & Canale's Nursing Care Planning Guides*. 8th ed. St. Louis, MO: Elsevier; 2020:806-844. [Context Link 1]
59. Montero-Odasso M, et al. World guidelines for falls prevention and management for older adults: a global initiative. *Age and Ageing* 2022;51(9):Online. DOI: 10.1093/ageing/afac205. [Context Link 1, 2] View abstract...
60. Scald Injury Prevention. Educator's Guide [Internet] American Burn Association. Accessed at: <https://ameriburn.org/>. [accessed 2023 Sep 12] [Context Link 1]
61. Greenberg SA. Chronic illness and older adults. In: Harding MM, Kwong J, Hagler D, Reinisch C, editors. *Lewis's Medical-Surgical Nursing: Assessment and Management of Clinical Problems*. 12th ed. St. Louis, MO: Mosby; 2023:63-80. [Context Link 1, 2, 3, 4]
62. Falls in Older People. NICE Quality Standard QS86 [Internet] National Institute for Health and Care Excellence. 2017 Jan Accessed at: <https://www.nice.org.uk/guidance/>. [created 2015; accessed 2023 Aug 08] [Context Link 1, 2, 3, 4]
63. Lam K, Shi Y, Boscardin J, Covinsky KE. Unmet need for equipment to help with bathing and toileting among older US adults. *JAMA Internal Medicine* 2021;181(5):662-670. DOI: 10.1001/jamainternmed.2021.0204. [Context Link 1, 2] View abstract...
64. Scott JM. Older adults. In: Rector C, Stanley MJ, editors. *Community and Public Health Nursing Promoting the Public's Health*. 10th ed. Wolters Kluwer; 2022:637-666. [Context Link 1, 2, 3]
65. Wellness nutrition. In: Grodner M, Escott-Stump S, Dorner S, editors. *Nutritional Foundations and Clinical Applications*. 8th ed. Elsevier; 2023:1-18. [Context Link 1]
66. Nutrition assessment and patient care. In: Grodner M, Escott-Stump S, Dorner S, editors. *Nutritional Foundations and Clinical Applications*. 8th ed. Elsevier; 2023:217-247. [Context Link 1, 2]
67. Harding MM. Obesity. In: Harding MM, Kwong J, Hagler D, Reinisch C, editors. *Lewis's Medical-Surgical Nursing: Assessment and Management of Clinical Problems*. 12th ed. St. Louis, MO: Mosby; 2023:1030-1049. [Context Link 1]
68. Dietary Guidelines for Americans, 2020-2025. 9th ed [Internet] U.S. Department of Agriculture and U.S. Department of Health and Human Services. 2020 Dec Accessed at: <https://www.dietaryguidelines.gov/>. [accessed 2023 Oct 17] [Context Link 1]
69. Guidelines for healthy eating. In: Dudek SG, editor. *Nutrition Essentials for Nursing Practice*. 9th ed. Wolters Kluwer; 2022:18-35. [Context Link 1, 2, 3, 4]
70. Water and minerals. In: Dudek SG, editor. *Nutrition Essentials for Nursing Practice*. 9th ed. Wolters Kluwer; 2022:125-152. [Context Link 1]
71. Nathan A. Malnutrition: impact on long-term conditions. *InnovAiT* 2021;14(4):263-265. DOI: 10.1177/1755738020919143. [Context Link 1, 2]
72. Gresh C, Taylor B. Promoting nutrition. In: Perry AG, Potter PA, Ostendorf WR, editors. *Nursing Interventions and Clinical Skills*. 7th ed. Elsevier; 2020:299-331. [Context Link 1]
73. Nutrition Education Resources and Materials. [Internet] U.S. Food and Drug Administration. Accessed at: <https://www.fda.gov/food/labelingnutrition/ucm20026097.htm>. Updated 2023 Sep [accessed 2023 Oct 18] [Context Link 1]
74. Food and supplement labeling. In: Dudek SG, editor. *Nutrition Essentials for Nursing Practice*. 9th ed. Wolters Kluwer; 2022:174-188. [Context Link 1]
75. Cadena-Malek Y. Communicable disease. In: Rector C, Stanley MJ, editors. *Community and Public Health Nursing Promoting the Public's Health*. 10th ed. Wolters Kluwer; 2022:204-241. [Context Link 1]
76. Murthy N, Wodi AP, McNally V, Cineas S, Ault K. Advisory committee on immunization practices recommended immunization schedule for adults aged 19 years or older - United States, 2023. *MMWR—Morbidity and Mortality Weekly Report* 2023;72(6):141-144. DOI: 10.15585/mmwr.mm7206a2. [Context Link 1] View abstract...
77. Recommended Adult Immunization Schedule for Ages 19 Years or Older, United States. Immunization Recommendations, United States, 2023 [Internet] Centers for Disease Control and Prevention. Accessed at: <https://www.cdc.gov/vaccines/schedules/hcp/imz/adult.html>. Updated 2023 Feb [accessed 2023 Oct 11] [Context Link 1, 2]
78. Bass AR, et al. 2022 American College of Rheumatology guideline for vaccinations in patients with rheumatic and musculoskeletal diseases. *Arthritis Care & Research* 2023;75(3):449-464. DOI: 10.1002/acr.25045. (Reaffirmed 2023 Mar) [Context Link 1] View abstract...
79. Sarasnick JA. Upper respiratory problems. In: Harding MM, Kwong J, Hagler D, Reinisch C, editors. *Lewis's Medical-Surgical Nursing: Assessment and Management of Clinical Problems*. 12th ed. St. Louis, MO: Mosby; 2023:577-595. [Context Link 1]
80. Standards of Practice for Case Management. [Internet] Case Management Society of America. 2022 Accessed at: <https://www.cmsa.org/>. [accessed 2023 Sep 21] [Context Link 1, 2, 3, 4]
81. Accreditation bodies, their standards, and requirements. In: Gillingham DC, editor. *Foundations of Case Management*. Blue Bayou Press; 2021:241-247. [Context Link 1, 2]
82. Standards and Guidelines for the Accreditation and Certification for Case Management Accreditation. Effective for Surveys Beginning on or After July 1, 2020 [Internet] National Committee for Quality Assurance (NCQA). 2020 Accessed at: To purchase copies of this publication, visit <https://ncqa.org>. [accessed 2023 Oct 24] [Context Link 1, 2]
83. Standards and Guidelines for the Accreditation of Case Management for Long-Term Services and Supports Program. Effective for Surveys Beginning on or After July 1, 2024 [Internet] National Committee for Quality Assurance (NCQA). 2023 Accessed at: To purchase copies of this publication, visit <https://ncqa.org>. [accessed 2023 Sep 05] [Context Link 1, 2]
84. Psychosis and Schizophrenia in Children and Young People: Recognition and Management. NICE Clinical Guideline CG155 [Internet] National Institute for Health and Care Excellence. 2016 Oct (NICE reviewed 2022) Accessed at: <https://www.nice.org.uk/guidance>. [accessed 2023 Aug 08]

[Context Link 1]

85. Guaiana G, et al. Cognitive behavioural therapy (group) for schizophrenia. Cochrane Database of Systematic Reviews 2022, Issue 7. Art. No.: CD009608. DOI: 10.1002/14651858.CD009608.pub2. [Context Link 1] View abstract...
86. Physical Activity Guidelines for Americans. [Internet] U.S. Department of Health and Human Services. 2nd ed; 2018 Accessed at: <https://health.gov/our-work>. [accessed 2023 Sep 18] [Context Link 1]
87. Keepers GA, et al. The American Psychiatric Association Practice Guideline for the Treatment of Patients With Schizophrenia. [Internet] American Psychiatric Association. 3rd ed; 2020 Accessed at: <https://psychiatryonline.org/>. [accessed 2023 Sep 21] DOI: 10.1176/appi.books.9780890424841. [Context Link 1, 2]
88. Nail RW, Herndon B, Mramba LK, Vogel-Anderson K, Hagen MG. An interprofessional primary care based transition of care clinic to reduce hospital readmission. American Journal of Medicine 2020;133(6):e260-e268. DOI: 10.1016/j.amjmed.2019.10.040. [Context Link 1] View abstract...
89. Nguyen HQ, et al. Association between post-hospital clinic and telephone follow-up provider visits with 30-day readmission risk in an integrated health system. BMC Health Services Research 2021;21(1):826. DOI: 10.1186/s12913-021-06848-9. [Context Link 1] View abstract...
90. Anderson A, et al. Follow-up post-discharge and readmission disparities among Medicare fee-for-service beneficiaries, 2018. Journal of General Internal Medicine 2022;37(12):3020-3028. DOI: 10.1007/s11606-022-07488-3. [Context Link 1] View abstract...
91. Marcondes FO, et al. Does scheduling a postdischarge visit with a primary care physician increase rates of follow-up and decrease readmissions? Journal of Hospital Medicine 2019;14:E37-E42. DOI: 10.12788/jhm.3309. [Context Link 1] View abstract...
92. Coppa K, Kim EJ, Oppenheim MI, Bock KR, Conigliaro J, Hirsch JS. Examination of post-discharge follow-up appointment status and 30-day readmission. Journal of General Internal Medicine 2021;36(5):1214-1221. DOI: 10.1007/s11606-020-06569-5. [Context Link 1] View abstract...
93. Bethishou L, Herzik K, Fang N, Abdo C, Tomaszewski DM. The impact of the pharmacist on continuity of care during transitions of care: A systematic review. Journal of the American Pharmacists Association 2020;60(1):163-177.e2. DOI: 10.1016/j.japh.2019.06.020. [Context Link 1] View abstract...
94. Medalie L, Cifu AS. Management of chronic insomnia disorder in adults. Journal of the American Medical Association 2017;317(7):762-763. DOI: 10.1001/jama.2016.19004. [Context Link 1] View abstract...
95. Smith DM, Friend C, Reeve J. Polypharmacy and rationalisation of medications. InnovAiT 2020;13(2):87-93. [Context Link 1]
96. Dietary Reference Intakes for Water, Potassium, Sodium, Chloride, and Sulfate. [Internet] National Academies. 2005 Accessed at: <https://nap.nationalacademies.org/>. [accessed 2023 Sep 18] [Context Link 1]
97. Ying YB, et al. Electroconvulsive therapy is associated with lower readmission rates in patients with schizophrenia. Brain Stimulation 2021;14(4):913-921. DOI: 10.1016/j.brs.2021.05.010. [Context Link 1] View abstract...
98. Gentry MT, Lapid MI, Rummans TA. Geriatric telepsychiatry: systematic review and policy considerations. American Journal of Geriatric Psychiatry 2019;27(2):109-127. DOI: 10.1016/j.jagp.2018.10.009. [Context Link 1] View abstract...
99. The case management process. In: Powell SK, Tahan H, editors. Case Management a Practical Guide for Education and Practice. 4th ed. Philadelphia, PA: Wolters Kluwer, Lippincott, Williams & Wilkins; 2019:92-118. [Context Link 1]
100. Drug therapy in older adults. In: Burchum JR, Rosenthal LD, editors. Lehne's Pharmacology for Nursing Care. 11th ed. Elsevier; 2022:102-107. [Context Link 1]

Footnotes

[A] Assess and address the emergency or urgent problems before continuing the engagement. [A in Context Link 1]

[B] A self-care plan tells you how to take care of your condition and when and where to get help for problems. [B in Context Link 1]

[C] Adherence in the early stages of the disease has an opportunity to influence its chronic and progressive course.(6) Early intervention in psychosis services can improve clinical outcomes, including admission rates, symptoms, and relapse rates, for people with a first episode of psychosis.(27) [C in Context Link 1]

[D] Electroconvulsive therapy (ECT) may be useful for psychotic symptoms, particularly if the patient is not responsive to pharmacologic treatment.(97) [D in Context Link 1]

[E] Cocaine, marijuana, and amphetamines can provoke psychotic symptoms in patients with schizophrenia.(8) [E in Context Link 1]

[F] Telehealth may improve access to and coordination of mental health care. Patients with behavioral health diagnoses may use telehealth or telepsychiatry to access mental health support or care (eg, to get assistance with problem-solving techniques or ask about presenting symptoms or medication use).(98) [F in Context Link 1]

[G] The timing of provider follow-up depends on the patient's disease severity, self-care complexity, patient/caregiver willingness and ability to provide patient care, and available social supports.(99) [G in Context Link 1]

[H] Neuroleptic malignant syndrome is a rare, rapidly progressive, potentially fatal complication more common with typical antipsychotics. Onset can occur hours or years after initiation of drug therapy.(41) [H in Context Link 1, 2]

[I] Tardive dyskinesia (TD) is more common with first-generation (typical) antipsychotics. The risk of TD is higher with long-term use of antipsychotics. The offending medication should be removed with the first sign of symptoms, as symptoms may be irreversible.(42) [I in Context Link 1]

[J] Use typical antipsychotic medication when psychotic symptoms outweigh the adverse effects of medication.(43) [J in Context Link 1]

[K] Fall risk factors include mobility or functional impairment, poor health, weakness, incontinence, polypharmacy (4 or more medications), medication dose changes, or patient taking substances that increase risk (eg, benzodiazepines, sedative-hypnotics, antidepressants, antipsychotics, alcohol, opiates).(64)(100) [K in Context Link 1]

[L] A pharmacist can support medication reconciliation for redundancy or duplication and identify drug interactions (eg, with other drugs, food).(61) [L in Context Link 1]

[M] Services to consider for community support include stairway modifications, widening of interior doors for ambulation equipment, and installation of handrails.(53) [M in Context Link 1]

[N] Adequate intake is used when the recommended daily allowance cannot be determined because of insufficient data on requirements.(69) [N in Context Link 1]

Codes

ICD-10 Diagnosis: F20.0, F20.1, F20.2, F20.3, F20.5, F20.81, F20.89, F20.9, F25.0, F25.1, F25.8, F25.9

DSM-5: F20.81, F20.9, F25.0, F25.1

MCG Health
Chronic Care 28th Edition
Copyright © 2024 MCG Health, LLC
All Rights Reserved

Last Update: 3/14/2024 1:47:40 AM
Build Number: 28.1.20240313235330.016020